

Victor Farmington Volunteer Ambulance, Inc.
emsCharts® Minimum Acceptable Documentation Standards

PURPOSE:

To define minimum acceptable standards for Victor Farmington Ambulance Personnel when documenting patient care using emsCharts®.

PROCEDURE:

Victor Farmington Ambulance personnel will adhere to the following standards when documenting patient care using emsCharts®.

GENERAL;

Chart must be started within 4 hours of calling in service and must be completed within 16 hours of calling in service.

Charts will be written using proper grammar, spelling and capitalization. Charts are not to be written in "caps lock" mode.

ABBREVIATIONS

The use of common medical abbreviations, while acceptable, should be limited. Personnel are encouraged to utilize plain English when writing charts, and to limit the use of abbreviations and acronyms. The use of the following abbreviations is prohibited:

U for unit	Write "unit"
IU for International Units	Write "International Units"
Trailing zero following a decimal point	Never write a zero by itself following a decimal point
Lack of leading zero before a decimal point	Always write a zero before a decimal point
MS; MSO ₄ ; MgSO ₄	Write "Morphine Sulfate" or "Magnesium Sulfate"
µg for "micrograms"	Write "mcg"

Creating a New Chart:

Victor Farmington Ambulance personnel are required to create a chart for any request for service that results in the generation of a call number. To create a new chart, the Primary Caregiver or crew member writing the chart in the event that no patient contact was made should log in to emsCharts®, select the “Patient Records” tab and then click on the “Create Blank Chart” button.

Create New Chart Page

Run number: Enter the Run Number of the call as assigned by VFVAC Dispatch.

VFVAC or FD Member?: Enter “Yes” if the patient transported is a member of Victor Farmington Volunteer Ambulance or the Victor, Farmington or Fishers Fire Departments or immediate family. Otherwise, enter “No”.

Basesite: Use the pick lists to select the location to which the crew was assigned when the request for service was received.

Unit: Use the pick list to select the unit number to which the crew was assigned and the level of care of the responding crew when the call was assigned.

Response: Use the pick list to select the type of response that was provided to the request for service.

Type of Svc.: Use the pick lists to select the type of service provided. For most responses the selection will be “Scene” and “Unscheduled”.

Category: Use the pick list to select the category of call that the dispatch information specifies.

Dispatched As: Use the pick list to select as closely as possible the type of problem indicated by the dispatch information.

Outcome: Use the pick list to select the appropriate description of the disposition of the request for service.

-ALS intercept with another transporting agency should be used when another service requests ALS and our ALS technician rides aboard another agencies ambulance.

-Cancelled should be used when you are cancelled before going en route to a call, en route to a call, or upon arrival by FD or PD.

-Field Termination should be used when the patient is declared dead after 30 minutes or more of resuscitative efforts on scene and is not transported.

- Obvious Death should be used when you arrive on scene and determine the patient is DOA
- Pt. evaluation only should be used when patient interaction has been made and a single service was provided without full assessment and treatment (Lift Assist only, MA checkover, etc).
- Stand By, no patient should be used for any event or fire stand-by that yields no patient contact.
- Treated BLS with ALS intercept from another agency should be used when ALS from any agency OTHER than VFVAC is utilized during transport
- Treated and Released after BLS Evaluation should be used when any incident where BLS treatment and/or evaluation was requested and transport was refused
- Treated and Released after ALS Evaluation should be used when the patient requests/requires ALS evaluation/Treatment by VFVAC is completed by VFVAC then refuses further treatment or transport. (Diabetic Wakeup, utilization of 4 lead EKG).
- Treated, transferred care to other medical care providers should be used when a VFVAC medic or crew transfers care to another VFVAC crew.
- Treated, Transferred to Air Medical should be used when VFVAC personnel transfer patient care to Air Medical Personnel.
- Treated, Transported ALS should be used when VFVAC transports a patient in the "ALS" mode. Putting the patient on a 4 lead EKG as only intervention is considered an ALS intervention and must be transported by the ALS provider.
- Treated, Transported BLS should be used when VFVAC transports a patient in the "BLS" mode.

Dispatch Loc.: Type in the location from which the crew responded to the request for service. Use VFVAC Base or Fishers Fire if responding from either of those base sites; otherwise use the actual location from where the crew responded.

Crew Members: Under the "D" line, use the pick list to select the name of the crew member who drove the ambulance to the HOSPITAL. Under the "P" line, use the pick list to select the name of the crew member who took care of the patient while en route to the hospital. The primary caregiver, as indicated by the "P", should always be the person who is writing the chart. In the event that the primary caregiver is a trainee the "S" (secondary caregiver) line should be used and be the preceptor monitoring the student or trainee. The "S" and "T" (third caregiver) line may also indicate any cleared provider that assists on a call as a second or third medic. In the event of a BLS or ALS student or observer, their name should be typed in the "other" box noting the certification level they are in class for. Anytime a student is a member of the crew the "student" and "preceptor" boxes should be checked appropriately.

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Requester: The Requester should always be Ontario County 911/ECD.

Destination Basis: Use the pick list to document why the destination hospital was chosen

Patient: VFVAC Personnel should follow the prompts given by EMS Charts and fill in all information in the following manner:

Relation Information

Last Name: Enter the surname of the patient. If unknown, click the “UNKNOWN” check box.

First Name: Enter the given name of the patient. If unknown, leave blank.

Middle Name: Enter the middle name or middle initial of the patient, if known

SSN: Enter the social security number of the patient. If unknown, leave blank.

Address: Enter the street number of the patient's address on the first line. Use the second line for apartment or unit number, as necessary. If unknown, type “Unknown” in the first line.

City: Enter the city of patient residence. If unknown, leave blank.

State: Enter the state of patient residence If unknown, leave blank.

Zip: Enter the zip code of the city of patient residence. Use the lookup feature to determine the patient's county and/or city of residence. If unknown, leave blank.

Sex: Enter the patient's gender

Height: Enter the numeric value of the patient height and select the measurement system that corresponds to the numeric value using the pick list. It is preferable to use the metric system when recording the patients height.

Weight: Enter the numeric value of the patient's weight and select the measurement system that corresponds to the numeric value using the pick list. It is preferable to use the metric system when recording the patient's weight.

Phone: Enter the patient's telephone number. If unknown, leave blank.

Consent Form Signed: Use the pick list to signify if the HIPPA form was completed.

Billing Information: Enter the name of the patients insurance and the policy number on the insurance card

Barriers to Care: Use the pick list to select any barriers to care encountered during care of the patient. If no barriers to care exist, select NONE. Do not leave barriers to care "NOT RECORDED".

DOB: Enter the patient's date of birth. EMS Charts should automatically calculate and fill in the patient's age. If unknown ?

Race: Enter the patient's race. If unknown, do not leave blank. Select "UNKNOWN" from the pick list.

Immunizations/Employer/Physician Buttons: If you have information regarding any of those items, click on the appropriate button and follow the prompts given by EMS Charts to enter the information. The PHYSICIAN button is to be used for entering information about the patient's personal physician, not the sending or receiving physicians.

Current PMHX: Use the pick list to select all conditions which the patient is currently under the care of a physician for or has been diagnosed with. If the condition is not found under the pick list, enter it manually. If the patient is not currently under the care of a physician or has no known medical conditions, click the red "X" box to enter none. If it is not known if the patient is under the care of a physician for any treatment, use the pick list to select "UNK".

Current Medications: Use the pick list to select all medications prescribed by a physician and/or all over-the-counter or homeopathic treatments that the patient is taking on a regular basis at home. If the medication or substance is not found in the pick list, enter it manually. If it is not known if the patient is currently taking any prescribed, OTC or homeopathic medications at home, use the pick list to select "UNK".

Allergies: Use the pick list to select any medications that the patient is allergic to. If the medication is not in the pick list, enter it manually. If it is unknown if the patient has any allergies to medications, use the pick list to select "UNK".

Obtained From: Use the pick list to select from whom the information regarding Current PMHX, Current Medications and Allergies was obtained.

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Symptoms: Use the pick list to document patients symptoms

Impression/Diagnosis: Use the pick list to select your impression of the patient's injuries or the diagnosis given by the sending facility.

Chief Complaint: Use the narrative to document the patient's chief complaint. This should be documented as a quotation using the patient's exact words. For example "my leg hurts!". If the patient has multiple complaints, try to ascertain the most severe complaint. If the patient is unable to speak, the chief complaint should be documented as follows: Pt. Non-Verbal S/P Multi-System Trauma, Pt. Non-Verbal S/P Cardiac Arrest, etc.

History of Present Illness: Use the narrative box to document all pertinent details surrounding the patient's present condition. Issues to address would include: SAMPLE History, OPQRST assessment and any other pertinent positives/negatives. Information included should be attributed to the appropriate source. For example: "pt. states that he last ate at approximately 1200 hours today", or, "family members state that pt. was complaining of a severe headache before she collapsed". Be as descriptive as possible in the HPI section. The HPI section should begin with the phrase "EMS requested to (dispatch location) for the (BLS/ALS) criteria call for the report of (a person with Chest Pain, MVC, etc), responded immediately".

Additional Injury Details/Cardiac Arrest/Drugs/Alcohol/Motor Vehicle Incident

Buttons: If pertinent information regarding additional details of the patient's injury is obtained, click on the Additional Injury Details button and follow the prompts given by EMS Charts to enter the data. If the patient was in cardiac arrest prior to or during our care, click on the Cardiac Arrest Button and follow the prompts given by EMS Charts to enter the data. The Drugs/Alcohol section should be filled out for every patient, as it is an important factor in determining the patient's condition. Click the Drugs/Alcohol button and follow the prompts given by EMS Charts to enter the data. Click on the Motor Vehicle Incident button and follow the prompts given by EMS charts for any patient involved an injury caused by a motor vehicle.

Scene Description: Use the narrative to describe the scene of the patient's injury or illness. Include details such as how you found your patient, what other agencies were on scene prior to your arrival and what care the other agency was performing upon your arrival.

NY Location Code: Use the pick list to select the town in which the call was located in

Patient Belongings: Use the narrative box to document any belongings transported with the patient and to whom they were turned over on arrival at the receiving facility. For example: cut clothes under patient, motorcycle helmet and boots TOT Dr. Idecwii and staff at receiving facility.

Patient Movement: Use the pick lists to document how patient was moved to vehicle, the patient position in the vehicle, how the patient was moved from vehicle, and reason for patient being on stretcher.

Factors Affecting Care: Use the narrative box to document any factors that directly or indirectly affected care of the patient.

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Neuro

Level of Consciousness: Use the pick list to select the descriptor which most accurately describes the patient's level of consciousness on initial contact.

Mental Present/Not Present: Use the pick list to select the descriptor that most accurately describes the patient's orientation upon initial contact.

Neurological Present/Not Present: Use the pick list to select the descriptor(s) which most accurately approximate the results of the patient's initial neurological examination.

Last Neurological Baseline Seen: Used to document the last approximate time a potential CVA patient was last seen normal.

Stroke Scale: Use the pick list to document the results of any stroke scale exam (Usually Cincinnati Stroke Scale)

Stroke/CVA Symptoms Resolved: Use the pick list to document if the stroke symptoms resolved and when the resolution occurred.

Comments: Use the narrative box to enter any comments pertinent to the patient's neurological status upon initial contact.

Patient Chemically Paralyzed: Use the pick list to indicate the patient's status in terms of chemical paralysis upon initial contact.

Loss of Consciousness: Use the pick list to indicate the whether the patient suffered a loss of consciousness at any time prior to arrival. If unknown, select "UNK".

Was Patient Immobilized? Use the pick list to indicate if the patient was immobilized prior to arrival.

Pupils: Use the pick list to document the size and reactivity of both the left and right pupil on initial contact. If unable to assess, select the appropriate pick list response.

Motor/Sensory: Use the pick lists to document motor and sensory response of all four extremities upon initial contact. If the patient is missing an extremity, select "NOT ASSESSED" and document the status of the extremity in the physical exam.

Airway

Status: Use the pick list to select the descriptor which most accurately describes the condition of the patient's airway upon initial contact. Do not leave the "STATUS" section blank.

Airway Exam: Use this pick list if any type of RSI exam or procedure is used. This box is used only by RSI providers

Secured via: Use the pick list to select the method by which the patient's airway is being secured upon initial contact. If the patient's airway is not secured upon initial contact, leave "SECURED VIA" blank and document same in the "COMMENTS" section.

Performed by: Use the pick list to select the type of provider who performed the maneuver that secured the airway.

Comments: Use the narrative box to document any pertinent information in regards to the patient's airway found on initial contact.

Tube Size: Document the internal diameter of the endotracheal tube used to secure the airway in millimeters.

Outcome: Use the pick list to select the patient's response to airway maneuvers performed prior to arrival.

Depth: Document the depth of the endotracheal tube at the level of the patient's teeth in centimeters.

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Respiratory

Effort: Use the pick list to select the descriptor that most accurately approximates the patient's respiratory effort on initial contact. Do not leave the "EFFORT" box blank.

Breath Sounds: Use the pick lists to select the descriptor that most accurately approximates the patient's breath sounds in both the right and left lungs on initial contact. Do not leave either "BREATH SOUNDS" box blank. Use the narrative box underneath the "BREATH SOUNDS" pick lists to add any additional documentation pertinent to the patient's breath sounds.

O2: Enter the amount of O2 the patient is receiving in l/min upon initial contact. If the patient is not receiving oxygen, leave blank.

Via: Use the pick list to select the device by which the patient is receiving oxygen. If the patient is not receiving oxygen, leave blank.

Performed by: Use the pick list to select the type of provider who placed the patient on oxygen. If the patient is not on oxygen, leave blank.

Outcome: Use the pick list to select the descriptor that most closely describes the patient's response to the administration of oxygen.

Comments: Use the narrative box to document any other pertinent information about the patient's respiratory status or response to the administration of supplemental oxygen.

Cardiovascular

Pulses: Use the pick lists to select the descriptor(s) that most closely approximate the quality of the patient's pulse(s) for each location where the pulse is assessed. If a pulse for a listed area is not assessed during patient contact, select "NOT CHECKED".

Temperature: Use the pick lists to document the patient's temperature and how the temperature was assessed.

JVD: Use the pick list to document the presence or absence of JVD on initial contact with the patient. If JVD is not assessed, select "NOT CHECKED".

Cap Refill: Use the pick list to document the patient's peripheral perfusion in terms of capillary refill time. If capillary refill is not assessed, leave blank.

Edema: Use the pick list to document any edema noted during initial assessment. If edema is not found, select "NOT APPRECIATED". If edema is not assessed for, select "NOT CHECKED".

Comments: Use the narrative box to document any other pertinent information regarding the patient's cardiovascular status on initial contact.

Pacemaker Button: If the patient is being treated with a pacemaker, either temporary or permanent, on initial contact, click the "PACEMAKER" button and follow the prompts given by EMS Charts to document the use of a pacemaker.

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Head

Assessment: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities that were found during the physical examination

Findings: Use the narrative box to expand on the findings during the physical exam of the head. Points to be addressed include:

- Presence or absence of deformity to the skull, facial bones and mandible
- Presence or absence of damage to the teeth
- Presence or absence of blood or CSF discharge from the ears or nose
- Presence or absence of periorbital ecchymosis
- Presence or absence of Battle's Sign
- Presence or absence of abnormal extraocular movements
- Color of the sclera
- Flaring of nostrils during respiration
- Examination of the anterior fontanelle (Infants)
- Description of any injuries found
- Presence of any facial droop or slurred speech
- Respiratory effort

Neck

Findings: Use the narrative box to document findings during the physical exam of the neck. Points to be addressed include:

- Condition of the neck veins
- Presence or absence of lymphadenopathy or masses
- Nuchal rigidity
- Use of accessory neck muscles during respiration
- Presence or absence of subcutaneous air
- Description of any injuries found

Chest

Assessment: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities that were found during the physical examination.

Findings: Use the narrative box to expand on the findings during the physical examination of the chest. Points to be addressed include:

- Presence or absence of: deformities, contusions, abrasions, penetrations, paradoxical movement, burns, tenderness, lacerations, swelling (DCAPP/BTLS)

- Presence or absence of asymmetrical chest movement

- Presence or absence of crepitus on palpation of the chest wall

- Presence or absence of subcutaneous air

- Presence or absence of accessory muscle use or sternal/intracostal retractions during respiration

- Presence or absence of lung sounds bilaterally and description of lung sounds

- Presence or absence of adventitious breath sounds

- Description of all injuries noted

Pelvis

Assessment: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities that were found during the physical examination.

Findings: Use the narrative box to expand on findings during the physical examination of the pelvic area. Points to be addressed include:

- Presence or absence of pelvic instability to lateral and A/P flexion

- Presence or absence of incontinence to urine/stool

- Presence or absence of blood at the meatus

- Presence or absence of rectal bleeding

- Presence or absence of femoral pulses

Abdomen

Left Upper/Right Upper/Left Lower/Right Lower: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities found during the examination of the abdomen.

Appearance: Use the narrative box to document the general appearance of the abdomen, i.e. round, flat, obese, etc.

Palpation: Use the narrative box to expand on findings during the physical examination of the abdomen. Points to be addressed include:

- Presence or absence of tenderness, rebound tenderness, guarding, rigidity

- Presence or absence of masses

Bowel Sounds: Description of the presence or absence and quality of bowel sounds.

Comments: Use the narrative box to expand on the findings of the abdominal examination. Points to be addressed include:

- Presence or absence of deformity, contusions, abrasions, penetrations, burns, trauma, lacerations, swelling (DCAP/BTLS)

- Presence of nausea and/or vomiting

- Description of any injuries found

Back

Spine: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities found during the examination of the spine. If the patient was placed in SMR PTA, select "NOT DONE".

Thoracic: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities found during the examination of the posterior thorax. If the patient was placed in SMR PTA, select "NOT DONE".

Lumbar: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities found during the examination of the lumbar area. If the patient was placed in SMR PTA, select "NOT DONE".

Findings: Use the narrative box to expand on findings during the physical examination of the back. If the patient was placed in SMR prior to arrival, document "PE of back deferred d/t pt. in SMR PTOA". Query providers who placed the patient in SMR regarding their findings of examination of the back and document same. If Victor Farmington Ambulance personnel perform physical examination of the back, points to be addressed include:

- Presence or absence of deformity, contusions, abrasions, penetrations, paradoxical movement, burns, trauma, lacerations, swelling (DCAPP/BTLS) for the posterior thorax, flanks and lumbar area.

- Description of any injuries found.

Extremities

LA/RA/LL/RL: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities found during the examination of the extremities.

Findings: Use the narrative box to expand on findings during physical examination of the extremities. Points to be addressed include:

- Gross motor and sensory function
- Presence or absence of peripheral pulses
- Lack of use of an extremity
- Presence or absence of deformity, angulation, discoloration, swelling
- Presence or absence of wounds

Restraints: Use the narrative box to document any restraints found on the patient during initial assessment.

Skin

Assessment: Use the pick list to select the descriptor(s) that identify any injuries or abnormalities found during the examination of the skin.

Findings: Use the narrative box to expand on the findings during the physical examination of the skin. Points to be addressed include:

- Skin color, temperature
- Skin turgor
- Presence of petechia, purpura, surgical scars
- Description of any injuries found

Additional Information

Burns/Drains & Tubes Buttons: To document additional assessment information pertaining to these specific topics, click the appropriate button and follow the prompts given by EMS Charts to enter the information.

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IVs Initiated Prior to Assessment

IVs: Enter all pertinent information into the boxes provided by EMS Charts for each IV established PTA. Fill in all boxes. This includes provider IVs and Hospital/Urgent Care staff IVs

Medications: Enter all pertinent information into the boxes provided by EMS Charts for each medication/infusion administered PTA. Fill in all boxes. This included Provider and Medical/Urgent Care staff medications.

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Activity Log

General: The Activity Log is meant to provide a time-referenced listing of all assessment and treatment activities taking place during a patient contact. Victor Farmington Ambulance personnel should follow Victor Farmington Ambulance Standards of Care during their treatment of each patient, and all actions and interventions are to be documented in the Activity Log, including – but not limited to: all recorded sets of vital signs, any invasive or non-invasive treatments provided, any medications administered and any and all patient response to medications or change in patient status.

Specific Requirements: While completion of the Activity Log is generally at the discretion of the Victor Farmington Ambulance personnel, the following specific entries are required:

Pt. Contact – The first line in the Activity Log documents patient contact. The time listed is the same time as the previously documented “At PT” time. Vital signs documented on this line will be vitals as received from the provider turning over care, if any. Victor Farmington Ambulance personnel will document under comments “Pt. contact – Vitals are PTOA”.

IV attempts: Document each attempt as a separate line item. The provider should document the location of attempt, size of catheter used, amount of fluid administered, and reason for failed attempt, if any.

Intubation Attempts: Document each attempt as a separate line item. The definition of attempt is anytime the ENDOTRACHEAL TUBE passes through the teeth. The provider should document the size of the tube, the size of the blade used, whether the vocal chords were visible, whether the tube was visualized passing the chords, methods the tube was confirmed and how the tube was secured. Use the pick lists provided to further document airway. If attempt failed, document reasons why

Cardiac Monitor: Initial 4 and 12 lead rhythms should be documented in comments section

Medications: Document each medication administration separately. Remember to include dose and rate. For Narcotic Administration remember to add lot number of medication

Medical Control: Document the hospital contacted using the drop down menu, name of physician spoken to, orders requested, orders received, and if denied reason why orders were denied if available should all be noted in the comments section.

Pt. Turnover – The last line in the Activity Log will document patient turnover. Vital Signs documented on this line will be a set taken by the receiving facility. Victor Farmington Ambulance personnel will document under comments “Pt. turnover – Vitals by receiving facility.” Additional documentation will include the name of the provider assuming care for the patient along with the fact that a full verbal report was given to receiving facility and the room in which the patient was placed. If the patient was placed in the waiting room Victor Farmington Ambulance personnel should document what condition the patient was left in and who directed Victor Farmington Ambulance personnel to place the patient in the waiting room.

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Miscellaneous Forms

State Export: Victor Farmington Ambulance personnel will complete the State Export form by clicking on “STATE EXPORT” and reviewing the State Export form for completeness. Victor Farmington Ambulance personnel should add any information to the State Export form that was not automatically imported from the patient chart by EMS Charts.

Special Reports: Victor Farmington Ambulance personnel should complete any required special reports for incidents occurring on a particular request for service such as in the even of an employee injury, vehicle damage, narcotics usage, AED use, RSI, or cardiac arrests.

Attached Files

Files: Victor Farmington Ambulance personnel will attach ancillary paperwork including – but not limited to – hospital face sheets, EKG tracings, Narcotic forms, Refusal of Treatment/Transport Forms and HIPAA Forms using the method and naming conventions specified by the IT department.

Signatures: Victor Farmington Ambulance personnel listed as “Primary Medic” will electronically sign the chart using their unique identifying number and password by following the prompts given by EMS Charts after clicking on “SIGN”.

Quality Assurance: After the “Primary Medic” has reviewed and signed the chart, the chart will be locked from further changes by clicking the “COMPLETE/LOCK CHART” button.

Addendums: If additional information needs to be added to the chart after it has been locked, Victor Farmington Ambulance personnel can attach an addendum by clicking on the “ADDENDUM” button.

“Default” buttons: Several sections of the EMS Charts patient care chart give the Victor Farmington Ambulance personnel completing patient care documentation the option of selecting “default” patient care documentation. Victor Farmington Ambulance personnel are strongly encouraged to use the “default” patient care documentation option with extreme care. This “default” setting provides minimal information about the patient and may not accurately reflect the patient's condition. If the “default” option is selected, Victor Farmington Ambulance personnel should carefully review this documentation to ensure that it accurately reflects the patient condition.